

HOSPITAL DISCHARGE SUMMARY

PATIENT DEMOGRAPHICS

Patient Name: Patient 24

Age: 81 years old | Sex: Female

MRN: 000-TEST-24

Admission Date: [Admission Date] | Discharge Date: [Discharge Date]

Facility: Metro Medical Center

PRIMARY DIAGNOSIS

Type 2 Diabetes Mellitus with Severe Hyperglycemic Episode & Acute Metabolic Derangement

SECONDARY DIAGNOSES

- Vascular dementia (moderate, baseline cognitive decline)
- Osteoporosis with history of vertebral compression fracture
- Hypertension (controlled)
- Hyperlipidemia
- Chronic kidney disease Stage 3b (eGFR 38)

HOSPITAL COURSE

Patient 24 presented via ED with complaints of polydipsia, polyuria, and generalized weakness × 4 days. Admission glucose was 487 mg/dL; HbA1c 11.2% (100 mmol/mol). Urinalysis positive for moderate ketonuria. Basic metabolic panel revealed creatinine 1.8, K⁺ 5.6 (mild hyperkalemia), bicarbonate 18 — consistent with euglycemic diabetic ketoacidosis

(euDKA) picture. Patient was initiated on IV regular insulin infusion at 0.1 U/kg/hr with concurrent D5 0.45 NS + 20 KCl at 250 mL/hr after stabilization of K+. Continuous glucose monitoring via POC fingerstick testing q1h × 12 hours, then q2h × 24 hours. Serum ketones trended downward by hospital day 2; metabolic acidosis resolved by day 3 (bicarb 24). Transition to basal-bolus insulin regimen initiated on day 3. Patient remained hemodynamically stable throughout admission; no episodes of hypoglycemia documented. Nephrology consulted given CKD Stage 3b; no acute tubular necrosis noted. Nutrition consulted for diabetic meal planning and tube-feeding protocol (patient has dysphagia history, currently po diet with supervision). Social work addressed placement concerns; patient will return to assisted living facility with 1:1 supervision.

DISCHARGE MEDICATIONS

Medication	Dose	Frequency	Duration
Insulin glargine (Lantus)	18 units	q.d. at bedtime	ongoing
Insulin aspart (NovoLog)	8 units	before meals (TID)	ongoing
Metformin ER	500 mg	q.d. *D/C'd — renal function*	
Lisinopril	10 mg	q.d.	ongoing
Atorvastatin	40 mg	q.d. at bedtime	ongoing
Alendronate sodium	70 mg	q.week (Mondays)	ongoing
Omeprazole	20 mg	BID	ongoing
Aspirin	81 mg	q.d.	ongoing

Glucose tablets/Fast-acting carbs PRN for hypoglycemia (BG < 70); alert facility staff immediately.

DISCHARGE CONDITION: **STABLE**

Activity & Weight-Bearing Status

Patient may resume ADLs with assistance. Fall precautions in place. No running, heavy lifting, or unassisted ambulation given osteoporosis and recent metabolic event. Supervised ambulation only.

DIETARY RESTRICTIONS

Diabetic, ADA 1800 kcal diet. No added sugars. Fluid restriction 1.5 L/day given mild CHF history. Soft/pureed consistency per swallow assessment;